

MERIDIAN & ASSOCIATES LLP

Advisory | Assurance | Tax | Consulting

Case Study: Post-Merger Integration

Regional Hospital System

CLIENT CONFIDENTIAL - BLINDED

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Meridian & Associates LLP | New York | Chicago | San Francisco | London | Singapore

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1. Engagement Overview

Client: 12-hospital regional system formed from merger of two health networks (name withheld)

Industry: Healthcare - Hospital Systems

Engagement Type: Post-Merger Integration - Clinical & Operational

Duration: 24 months (July 2023 - June 2025)

Team Size: 120+ professionals at peak staffing

Total Fees: \$38.4 million

Lead Partner: Michael Torres, CPA, PMP, FACHE

Meridian Offices: Chicago (lead), New York, Hyderabad (GDC)

2. Client Situation & Challenge

The client was formed through the merger of two established regional health networks, creating a combined system of 12 hospitals, 185 ambulatory care sites, 4,200 physicians, and 38,000 employees serving a metropolitan area of 3.2 million residents. The merger, valued at \$4.8 billion, was driven by the need to achieve scale, improve clinical outcomes, and realize significant cost synergies in an increasingly competitive and value-based healthcare market.

The newly formed system faced several critical integration challenges:

- **Duplicate EHR Systems:** Network A operated on Epic (since 2016, fully integrated across 7 hospitals) while Network B used Cerner Millennium (deployed in 2014 across 5 hospitals). Maintaining two EHR platforms was projected to cost \$28M annually in redundant licensing, support, and interface maintenance, and created patient safety risks at cross-network referral points where records were not accessible.
- **Clinical Workflow Variation:** The two networks had independently developed clinical protocols, order sets, formularies, and documentation standards over decades. Initial assessment identified 2,400+ clinical workflow variations across 180 clinical departments, with 340 classified as high-priority due to patient safety implications.
- **Synergy Target Pressure:** The merger business case committed to \$45M in annual run-rate synergies within 30 months, including \$18M from IT consolidation, \$15M from supply chain standardization, and \$12M from revenue cycle optimization. The Board and investor community were closely monitoring progress against these targets.
- **Workforce Integration:** Cultural differences between the two networks were significant. Network A was an academic-affiliated system with a research-oriented culture, while Network B was a community-based system focused on operational efficiency. Physician alignment and retention were critical risks, particularly among high-revenue surgical specialties.
- **Regulatory Requirements:** The merged entity needed to file updated Medicare and Medicaid provider enrollment, consolidate compliance programs, and ensure HIPAA-compliant data sharing across the newly unified system, all while maintaining CMS Conditions of Participation at every facility.

3. Meridian's Solution

Meridian designed and executed a comprehensive post-merger integration program organized into four interconnected workstreams, each with dedicated leadership and measurable milestones:

3.1 Unified Epic Implementation

After a thorough evaluation, the steering committee selected Epic as the unified EHR platform, leveraging Network A's mature Epic environment as the foundation. Meridian led the extension of Epic to Network B's 5 hospitals and 85 ambulatory sites through a phased rollout: three community hospitals in Wave 1, followed by two specialty hospitals in Wave 2. The implementation included Epic Beaker (laboratory), Radiant (imaging), Willow (pharmacy), and Cadence (scheduling), achieving a single patient record across all 12 facilities.

A dedicated data migration team converted 4.2 million patient records and 12 years of clinical history from Cerner to Epic, using a custom migration toolkit developed by Meridian's GDC team. The migration achieved 99.98% data integrity as validated by an independent clinical data audit.

3.2 Clinical Workflow Harmonization

Meridian deployed a clinical integration team of 35 professionals (including 8 clinicians with direct patient care experience) to harmonize clinical workflows across the merged system. The team facilitated 220 clinical governance sessions involving 600+ physicians and clinical leaders to develop unified order sets (reduced from 4,800 to 1,200), a single formulary (consolidated from two formularies with 62% overlap), and standardized clinical documentation templates for all major service lines.

3.3 Shared Services Center for Revenue Cycle

Meridian designed and stood up a centralized Revenue Cycle Management (RCM) shared services center serving all 12 hospitals. The center consolidated previously distributed functions including patient access/registration, coding, billing, claims submission, denial management, and collections. Key improvements included:

- Centralized coding pool with CAC (Computer-Assisted Coding) technology, improving coding accuracy from 88% to 96%
- Automated claims scrubbing engine reducing clean claims rejection rate from 18% to 4%
- Dedicated denial management unit with root cause analytics, reducing denial rate from 12.4% to 9.7%
- Single patient financial services model with unified pricing transparency and financial counseling
- Standardized charge capture process reducing revenue leakage by an estimated \$8.2M annually

3.4 IT Infrastructure Consolidation

The IT consolidation workstream addressed the merger of two independent data centers, network infrastructure, and end-user computing environments. Key activities included data center consolidation from 4 facilities to 2 (primary + disaster recovery), network interconnection via dedicated MPLS circuits, Active Directory forest merge, and unified endpoint management for 28,000 devices. The team also consolidated 14 redundant clinical applications, reducing annual application licensing costs by \$6.8M.

4. Delivery Approach & Team Composition

The 24-month program was structured into three phases:

- **Phase 1 - Foundation (Months 1-6):** Integration planning, governance structure, EHR platform decision, clinical workflow assessment, Day 1 readiness activities
- **Phase 2 - Execution (Months 7-18):** Epic Wave 1 and Wave 2 deployments, shared services center buildout, IT infrastructure consolidation, clinical workflow harmonization
- **Phase 3 - Optimization (Months 19-24):** Performance optimization, benefits realization tracking, knowledge transfer, transition to steady-state operations

Team composition at peak staffing (120+ professionals):

- 2 Partners, 6 Senior Managers, 12 Managers, 32 Senior Consultants, 40 Consultants, 28 Analysts
- 8 embedded clinicians (3 physicians, 3 RNs, 2 pharmacists) for clinical workflow design
- 35,000 end users trained through a blended learning model (e-learning, classroom, at-the-elbow support)
- Dedicated OCM (Organizational Change Management) team of 15 professionals

5. Results & Impact

Metric	Baseline	Result	Improvement
Synergies Realized (annual)	\$0 (pre-merger)	\$52M	116% of \$45M target
Claims Denial Rate	12.4%	9.7%	22% reduction
Patient Record Unification	Dual systems	Single EHR	100% unified

Physician Satisfaction	76%	82%	Maintained >80%
Days in A/R	48.2 days	38.6 days	20% reduction
IT Application Portfolio	280 applications	196 applications	30% consolidation
Annual IT Operating Cost	\$62M combined	\$44M unified	\$18M savings

6. Client Testimonial

"The Meridian team understood from day one that this was not just a technology project -- it was a clinical transformation. Their ability to engage our physicians and nursing staff in the design process, combined with their operational rigor in managing a program of this complexity, was exceptional. We exceeded our synergy targets while maintaining clinical quality and physician satisfaction, which is the true measure of a successful integration."

-- Chief Executive Officer, Client Health System